Working Manual for Investigators

ICMR-Funded Project on Comprehensive ICU–Palliative Care Integration

1. Purpose of This Manual

This manual serves as an operational guide for investigators newly joining the project titled:

"Implementing a Multifaceted, Comprehensive, Appropriate Care Package for People with Advanced and Terminal Illnesses Needing ICU Support and Assessing Its Impact on Quality of Dying, Family Satisfaction, Care Cost, and Resource Utilisation."

It outlines investigator roles, study procedures, ethical responsibilities, data management processes, and reporting expectations to ensure standardisation, protocol adherence, and data integrity.

2. Overview of the Project

Project Start Date

15 September 2025

Funding Agency

Indian Council of Medical Research (ICMR)

Nature of Study

Implementation / interventional health systems research integrating palliative care into ICU settings.

Primary Outcome Domains

- Quality of dying and death
- Family satisfaction with care and communication
- Cost of care

- ICU resource utilisation

3. Investigators: Roles and Responsibilities

3.1 Principal Investigator (PI)

- Overall scientific and administrative oversight
- Final authority on protocol adherence, ethical issues, and reporting
- Liaison with ICMR and institutional authorities

3.2 Co-Investigators

- Supervision of specific domains (clinical implementation, data analysis, training)
- Quality assurance of data and intervention fidelity

3.3 Newly Onboarded Investigators

You are expected to:

- Understand the study protocol thoroughly
- Implement assigned components consistently
- Ensure ethical standards and patient dignity
- Maintain accurate and timely documentation

4. Study Population and Eligibility

Inclusion Criteria

- Patients with advanced or terminal illness admitted to ICU
- Patients/families for whom goals-of-care discussions are appropriate

Exclusion Criteria

- ICU admissions with expected full recovery
- Lack of consent where applicable

Investigators must strictly adhere to approved inclusion/exclusion criteria.

5. Description of the Care Package

The intervention consists of a multifaceted palliative care integration, including:

- Early identification of patients with serious illness
- Structured communication with families
- Symptom assessment and management
- Shared decision-making and goals-of-care discussions
- Coordination between ICU and palliative care teams

All interventions must be delivered as per protocol, without deviation unless clinically justified and documented.

6. Research Methods Used in This Project

This project employs a mixed-methods, implementation-focused research design, combining quantitative and qualitative approaches to evaluate both outcomes and processes of integrating palliative care into ICU settings.

The methods are chosen to capture:

- Measurable changes in care and resource use
- Experiential outcomes for patients and families
- Feasibility and acceptability of the intervention in real-world settings

6.1 Overall Study Design

Prospective, interventional, mixed-methods implementation study

Key Characteristics:

- Conducted in real-world ICU settings
- Focused on *how* care is delivered, not only outcomes
- Evaluates both effectiveness and process fidelity

6.2 Quantitative Research Methods

6.2.1 Outcome Measurement

Used to objectively assess the impact of the care package.

Outcomes Studied:

- Quality of dying and death
- Family satisfaction
- ICU length of stay
- Resource utilisation
- Cost of care

How Investigators Apply This in the Field:

- Identify eligible patients early in ICU admission
- Apply standardised assessment tools as per protocol
- Ensure data are recorded prospectively, not retrospectively
- Complete forms in a timely, factual, and unbiased manner

Important:

Investigators must never “estimate” or “infer” outcomes. If data are unavailable, record as missing and report.

6.2.2 Health Economics and Resource Utilisation

This method evaluates whether integrated palliative care leads to:

- Reduced non-beneficial interventions
- More appropriate ICU utilisation

Field Application:

- Extract ICU resource data from patient records:
    - Ventilator days
    - Investigations
    - Interventions
- Collaborate with billing/accounts sections where required
- Ensure consistency across cases

6.3 Qualitative Research Methods

6.3.1 Family Satisfaction and Experience

This captures the lived experience of families, which cannot be reduced to numbers alone.

Method:

- Structured or semi-structured questionnaires/interviews
- Conducted with sensitivity and appropriate timing

Field Application:

- Approach families after critical discussions or at an appropriate emotional moment
- Use non-directive, neutral language
- Avoid leading questions
- Record responses verbatim where possible

Key Principle:

Qualitative data collection is not "casual conversation"; it is a structured research activity requiring discipline and empathy.

6.3.2 Process Evaluation

This examines *how* the care package is implemented.

Examples:

- Was palliative care initiated early?
- Were goals-of-care discussions documented?
- Was interdisciplinary communication effective?

Field Application:

- Document:
  - Timing of palliative involvement
  - Participants in discussions
  - Decision-making processes
- Note deviations from protocol with reasons

6.4 Implementation Science Methods

This project explicitly evaluates feasibility, acceptability, and sustainability.

Field Indicators:

- Team acceptance of the care package
- Barriers (staffing, time, culture)
- Facilitators (training, leadership support)

Investigator Role:

- Observe and document contextual factors
- Provide reflective notes during regular reviews
- Avoid blaming perspectives; focus on systems and processes

6.5 Ethical Research Methods in End-of-Life Contexts

This study requires ethically-attuned research conduct.

Field Principles:

- Consent may be dynamic and ongoing
- Respect patient and family values
- Avoid therapeutic misconception
- Distinguish clearly between clinical care and research documentation

Investigators must escalate ethical uncertainties immediately to the PI.

6.6 Integration of Methods in Daily ICU Practice

Investigators should understand that:

- Clinical work and research overlap but are not identical
- Research documentation must never delay or compromise care
- Observational accuracy is as important as clinical skill

Practical Example:

A goals-of-care meeting is:

- A clinical intervention → requires empathy
- A research process → requires documentation

Both roles must be fulfilled.

6.7 Common Field Errors to Avoid

- Retrospective completion of forms
- Over-interpretation of qualitative responses

- Deviations from protocol without documentation
- Recording personal opinions instead of observations

6.8 Common Field Situations and Practical Responses

Situation	Recommended Investigator Response
Family members become emotionally distressed during discussion	Pause the interaction respectfully and allow time before continuing
ICU staff are occupied with active patient care	Revisit later and avoid interrupting clinical work
Family requests medical advice beyond investigator role	Redirect questions to the treating clinical team
Conflicting opinions are expressed by different team members	Avoid taking sides and document only observable processes
Family declines participation	Respect the decision without repeated persuasion
Timing appears inappropriate for discussion	Delay approach and reassess later with ICU team guidance

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7. Ethical Conduct and Patient Dignity

Investigators must:

- Respect patient autonomy and family values
- Ensure non-coercive participation
- Maintain sensitivity during end-of-life discussions
- Escalate ethical concerns immediately to the PI

Compassionate conduct is as important as scientific accuracy.

8. Communication and Documentation Standards

- All case discussions should be documented clearly
- Communication interventions must be noted (date, participants, content summary)
- Adverse events or conflicts must be reported promptly

9. Monitoring, Quality Assurance, and Audits

- Regular internal review of data completeness
- Random verification of records
- Compliance with ICMR audit and reporting requirements

Investigators may be asked to clarify data during monitoring or during final reporting.

10. Reporting and Dissemination

- Investigators may contribute to manuscripts, abstracts, or reports
- Authorship will follow ICMR and ICMJE guidelines
- No data may be shared externally without PI approval

11. Professional Conduct

New investigators are expected to:

- Attend project meetings and training sessions
- Maintain professionalism with patients, families, and team members
- Declare any conflicts of interest

12. Points of Contact

- **Principal Investigator:** Overall guidance and escalation
- **Project Coordinator (if applicable):** Day-to-day operational issues
- **Ethics Committee:** Ethical concerns